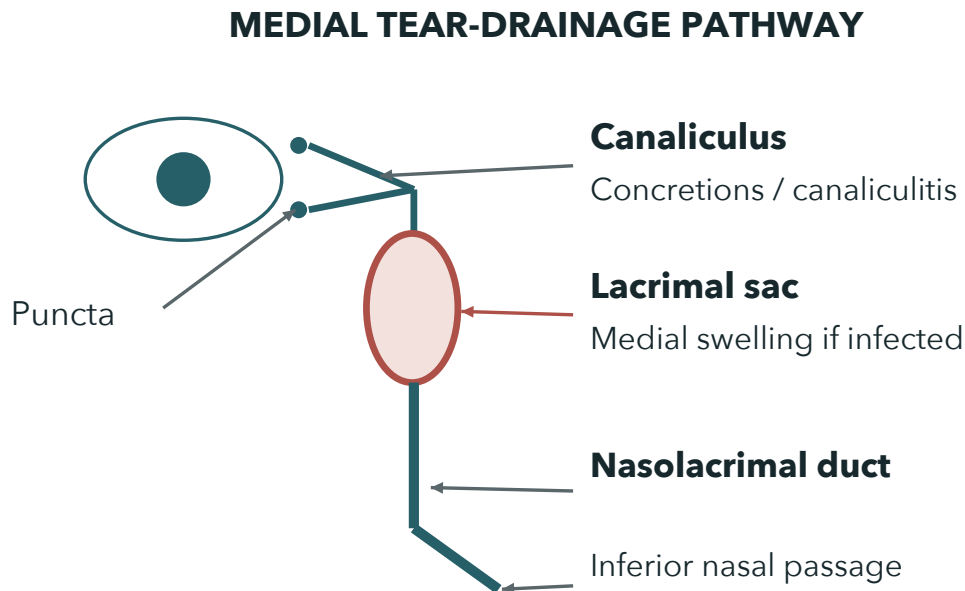


Tear drainage: obstruction and infection

CLINICAL TYPE B | Focused decisions for M3-M5

Localise the centre of the problem

Three anatomically different sites



Neonatal cyst extension can obstruct breathing

Original tear-drainage schematic; the globe is shown to orient the medial puncta.

Location distinguishes the presentation

Finding	Clinical inference
Pouting punctum + concretions	Canaliculitis; retained material may require specialist removal.
Tender swelling below medial canthus	Lacrimal-sac infection; assess systemic illness/spread.
Infant tearing with clear comfortable cornea	Possible NLDO; photophobia or a large/cloudy cornea changes the differential.

References AAO: Canaliculitis | AAPOS: Tear-duct obstruction

Choose the next drainage decision

Uncomplicated congenital obstruction

- 1** At any age: recurrent infection, severe symptoms or an atypical examination → earlier specialist reassessment.
- 2** If uncomplicated: appropriate massage and observation during infancy; drops only control significant discharge.
- 3** Persistent symptomatic disease beyond infancy → discuss specialist probing; treatment and anaesthesia depend on the child.

Dacryocystocele: check the nose

Finding	Clinical inference
Blue medial swelling	May reflect obstruction at both ends with a distended sac.
Noisy breathing / feeding distress	Intranasal cyst extension can compromise neonatal breathing, especially bilaterally. Urgent airway + ophthalmology/ENT assessment.
Infected swelling	Systemic antibiotics and hospital care may be necessary.